



OFFICIAL

NMHS MHS PROCEDURE

Safe Use of Low Stimulus Environments

Scope (Audience)	All NMHS Mental Health Service (MHS) Workers
Scope (Area)	All NMHS MHS Inpatient Areas
Summary	To promote the safe and appropriate use of Low Stimulus Environments such as Quiet Rooms or designated areas within inpatient settings available to patients upon request.
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Standard 1) Partnering with Consumers (Std 2)
Policy Sponsor	Adult Inpatient Operational Co Director
Policy Contact	Inpatient Co Directors
Version	3.0
Effective Date	18/12/2025
Review Date	18/12/2028

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

To promote the safe and appropriate use of Low Stimulus Environments such as Quiet Rooms or designated areas within inpatient settings available to patients upon request. Safe use of Low Stimulus Environments should be in accordance with the Consumer's Safety Plan and their Treatment Support and Discharge Plan (TSDP) on the Psychiatric Service On-Line Information System (PSOLIS).

The use of low stimulus environments will be consistent with the best available evidence, recovery principles, therapeutic engagement, cultural needs, individualised and trauma-informed care.

2. Background

A 'low stimulus environment' refers to a room or allocated space in an inpatient setting identified as a quiet area. The area should be utilised for patients who request a low stimulus area as a therapeutic intervention or strategy to reduce stress levels, engage in relaxation, sensory modulation and other self-soothing techniques possibly practiced during therapy sessions with nursing staff and other members of the multidisciplinary team (MDT). It may also be suggested by workers as a positive strategy for the patient to use.

The [Chief Psychiatrist of Western Australia \(WA\) Guidelines](#) identify that a MDT approach is essential to ensure that individuals with multiple complex needs have access to the broad range of evidence-based interventions available to support treatment and recovery. Providing access to low stimulus environments in inpatient mental health settings in addition to other therapeutic interventions has been shown to increase patient satisfaction with the hospital experience ([The University of Western Australia – Clinical Guidelines for the Physical Care of Mental Health Consumers](#)). In accordance with the [NMHS Carers Policy](#), and the [Western Australian Carers Charter](#), consideration should also be given to involvement of relevant others to support individuals, such as the [MHS Aboriginal Liaison Service](#), Peer support workers, Carers, or other Personal support persons.

While *Pro re nata* (PRN) medication can be used to reduce emotional and behavioural disturbances and manage distress, over-reliance on PRN medication and other restrictive practices can promote dependency and prevent patients from developing more functional coping skills. Identifying more therapeutic options in collaboration with patients promotes therapeutic engagement and encourages patients to regain control of their emotions and behaviours and learn skills that can be generalised to other situations after discharge. Refer to **Appendix 1: Clinical Aid Map** – Interventions under the themes of physical health, relaxation, distraction, environment and psychological health.

Patients have emphasised that it is important that there is “*meaningful activity for them to engage in to distract them when they feel overwhelmed or distressed*”. Providing access to a low stimulus area provides patients with a safe space to relax, listen to their favourite music or engage in an activity of choice. Where possible all patients in WA mental health facilities will have access to low stimulus environments. Patients accessing low stimulus areas will be monitored by allocated nurses and other clinicians to ensure that they are safe and the space is respected as a quiet area in accordance with the requirements of the [MHS Clinical Risk Assessment and Management Policy](#).

3. Risk

Adherence to this procedure is reflective of the needs of inpatients within a mental health setting, as part of recovery pathways that are trauma informed and gender sensitive and recognise common needs of individuals for privacy and personal space.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☒	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Therapeutic Environment

A therapeutic environment is recognised by the following:

- Demonstrates the core values of the organisation: care (Kaaradjiny), respect (Ngargal-wirrn), innovation (Milka Kaaditj), teamwork (Danjoo Yacker) and integrity (Karnadjil) that are evident in all aspects of service delivery.
- Promotes therapeutic engagement and patient safety through individualised, culturally safe, trauma-informed and recovery focused care.
- Cultural awareness and sensitivity for an individual's social, cultural and spiritual background and beliefs must be considered. Access to appropriate interpreting and translating services or Aboriginal Liaison Officers are to be provided where appropriate to the patient and carers.
- Acknowledgement of the vulnerability of patients due to past history of illness, trauma and other negative experiences.
- Recognises trauma arising from the use of restrictive practices such as restraint and seclusion and the potential for re-traumatisation.
- Recognises the need for routine identification of risk (including sexual safety risks) while balancing personal autonomy and decision making with the requirement to exercise a duty of care.
- Engages with patients, family, carers and others involved in patient care planning that is collaborative and inclusive.

5. Principles

- All staff are required to comply with the requirements of the [Mental Health Act 2014](#), the Chief Psychiatrist's [Standards for Clinical Care](#); the [National Safety and Quality Health Service Standards](#) and mandatory policy and reporting requirements.
- Patients have a right to receive care that is appropriate to their needs and is consistent with their rights under the *Mental Health Act 2014* and the [Australian Charter of Healthcare Rights](#), including access to culturally appropriate program/s, an Aboriginal mental health worker/s and direct care that is appropriate for people within the Culturally and Linguistically Diverse (CaLD) community.

- The use of a low stimulus area is a contemporary practice to provide a safe and therapeutic area as a preferred method of intervention, to assist patients to maintain emotional regulation and reduce the stress and anxiety associated with hospitalisation.
- Access to designated low stimulus areas has demonstrated increased patient engagement with mental health services and provides a resource that promote safety and collaboration with care planning during hospitalisation.
- Access to quiet areas is a safe space that provide opportunities for patients to reduce stress and regain emotional regulation, maintaining their dignity, personal autonomy and in some acute environments has been shown to reduce incidents of restraint and seclusion ([Safe Wards Report](#)).
- The use of a low stimulus area should be consistent with the patient's care plan and coping strategies. Refer to the [Consumer Safety Plan](#) (SMHMR932)
- All patients accessing the low stimulus area will have a Risk Assessment and Management Plan (RAMP) in place.
- Assessment will be undertaken as per the [SSCD Mental Health Framework](#) and the requirements of the [MHS Clinical Risk Assessment and Management Policy](#).
- Consideration must be given to the use of safe and appropriate resources within the area to ensure further risks are not introduced.

6. Processes

6.1 Equipment and Resources

All workers required to monitor/observe patients in low stimulus areas will have access to the environment and equipment when required including safety features such as duress alarms, emergency call buttons, visibility panels or line of sight depending on physical structure or geographical design.

Patients will have access to:

- Comfortable furniture, with soothing décor and ambient lighting that can be adjusted depending on preferences and requirements.
- Sensory trolley with a range of therapeutic aids and equipment including music, soothing sounds and stress reducing, headphones, weighted blankets, soft furnishings (individual pillows/cushions) comfortable loungers, bean bags and other relaxation aids.
- Resources to connect Aboriginal patients back to country (visual aids, music).

Storage and cleaning schedules will be in place for equipment that meet Infection Prevention and Control requirements ([Infection Prevention and Control Policy](#)).

6.2 Use of low stimulus areas as a therapeutic intervention

- Patients should be orientated to the location of a low stimulus area on admission as part of their admission, this may need to be revisited at regular intervals.
- Access to the area may be requested by a patient in the knowledge that they are free to leave this area at any time.
- Access may be suggested to the patient by the worker as a therapeutic option. If the area is in use, an alternative may need to be considered or agreed upon.
- The allocated nurse for the patient must be involved in the decision-making process.

- Discussion between the worker and patient will include identification of the potential benefits of having a quiet area with access to sensory modulation equipment (weighted blanket, stress balls, relaxation video/earphones).
- The use of this strategy is recorded on the patient's Coping and Safety Tool for Awareness and Recovery (PMR 10).
- The allocated nurse must inform the shift coordinator when a patient accesses a low stimulus area so that clinical observation requirements are maintained as per the TSDP and recorded in the medical record by the allocated nurse.
- Documentation will include observations, mental state assessment, and patient self-report of benefits and outcomes from the use of the low stimulus area.
- The frequency of clinical observations will depend on the TSDP, clinical presentation and identified risk factors at the time.
- Consideration should also be given to the inclusion of relevant others to support individuals, such as the Aboriginal Health Liaison Service, Peer support workers, Carers or other Personal support persons.
- Observations will be recorded in the Visual Observation Chart (PMR 29) and clinical record in accordance with [MHS Clinical Risk Assessment and Management Policy](#).

6.3 Safety concerns

If safety concerns are identified, the nurse will:

- Review the therapeutic interventions and if possible, seek information from the patient on supportive options available to address areas of concern. Check the Consumer Safety Plan (SMHMR932) and TSDP and follow agreed plan of action if identified.
- Undertake a Mental State Assessment as per the [MHS Clinical Risk Assessment and Management Policy](#) and [MHS Recognising and Responding to Acute Deterioration \(RRAD\) Physiological and Mental State Procedure](#) and escalate care as required by notifying the shift Co-ordinator, Senior Nurse and Medical Officer.
- Interventions may include use of counselling skills and supportive de-escalation strategies to promote emotional regulation, consulting with the Medical Officer and arranging for a patient review including consideration of PRN medications.
- Update physical observations where required on the Observation & Response Chart (ORC) and record PRN Medication Chart in accordance with the [MH Recognising and Responding to Acute Deterioration \(RRAD\) Physiological and Mental State Procedure](#)
- Update the RAMP and additional mental health tools as indicated in accordance with the [MHS Clinical Risk Assessment and Management Policy](#) and SSCD requirements.
- Provide ongoing reassurance for the patient, and notify family, carer and personal support person with the consent of the patient.
- Update the TSDP and RAMP in PSOLIS and document in the Progress Notes in the Medical record.
- Update information in iSOFT and utilise the ISoBAR clinical handover framework in clinical handovers in accordance with the Department of Health's [Clinical Handover Policy](#) and the [Clinical Handover Matrix](#).

7. Compliance, monitoring and Evaluation

Compliance with this procedure is mandatory.

- All clinicians attending orientation to the clinical environment will receive information regarding the use of the Low Stimulus areas and the procedures for use.
- Incidents relating to the use of low stimulus areas will be recorded on Datix-CIM and will be reviewed at service Leadership Committees.
- Consumer feedback surveys can be used to determine consumer satisfaction with access to Low Stimulus areas.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed every three (3) years using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

8. Definitions

Terms	Definitions
Low stimulus environment	Refers to a safe space that is removed from shared amenities with an ambient décor that most people find relaxing, and an individual may request access to this area with staff supervision. This does not include an open seclusion room.
Therapeutic engagement	Is when supportive and engaged health professionals demonstrate the therapeutic principles of respect, collaboration, consensus, optimism and recovery in all interactions with patients/consumers.

9. Related Documents

WA Health mandatory policies	• DOH State-wide Standardised Clinical Documentation Policy • MP 0095/18 Clinical Handover Policy • Clinical Handover Matrix • MP 0181/24 Safety Planning for Mental Health Consumers Policy
NMHS Policy documents	• NMHS Clinical Incident Management Policy • NMHS Clinical Documentation Policy • NMHS Infection Prevention and Control Policy
NMHS MHS Policy documents	• MH Clinical Risk Assessment and Management Policy • MH Recognising and Responding to Acute Deterioration Procedure • MH Code Blue Medical Emergency Procedure – Inpatients • MH Management of Clinical Alerts Policy • MH Medication: Prescribing Administration and Management for Inpatients Policy • MHSDHS Mandatory Training Policy

Office of the Chief Psychiatrist	• The Chief Psychiatrist Clinical Guidelines 2015 • The Chief Psychiatrist Clinical Standards 2015 • The Chief Psychiatrist Sexual Safety Guidelines 2020 • The Chief Psychiatrist Report, People with severe enduring mental illness and complex needs including those with challenging behaviour. A targeted review of adult public mental health services in metropolitan Perth, 2020
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10. Document Control

Document Version Control			
Date	Version	Author	Notes
09/10/2013	1.0	MHEG	Initial endorsement
05/10/2015	1.1	A/Executive Director	Minor update
08/06/2022	2.0	Nurse Director MHPHDS	Scheduled review, incorporating changes in practice, equipment, governance, audits and safety. Implementation of changes as directed by the ACSQHC to include Recognition and Responding to Mental Health Deterioration, update of template and links
18/12/2025	3.0	MHS Inpatient Co Directors	Scheduled review completed, updates include Legislative and Department of Health policies, Equipment, & resources reviewed; duress information, safety concerns and documentation requirements revised. Template and hyperlinks refreshed.

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

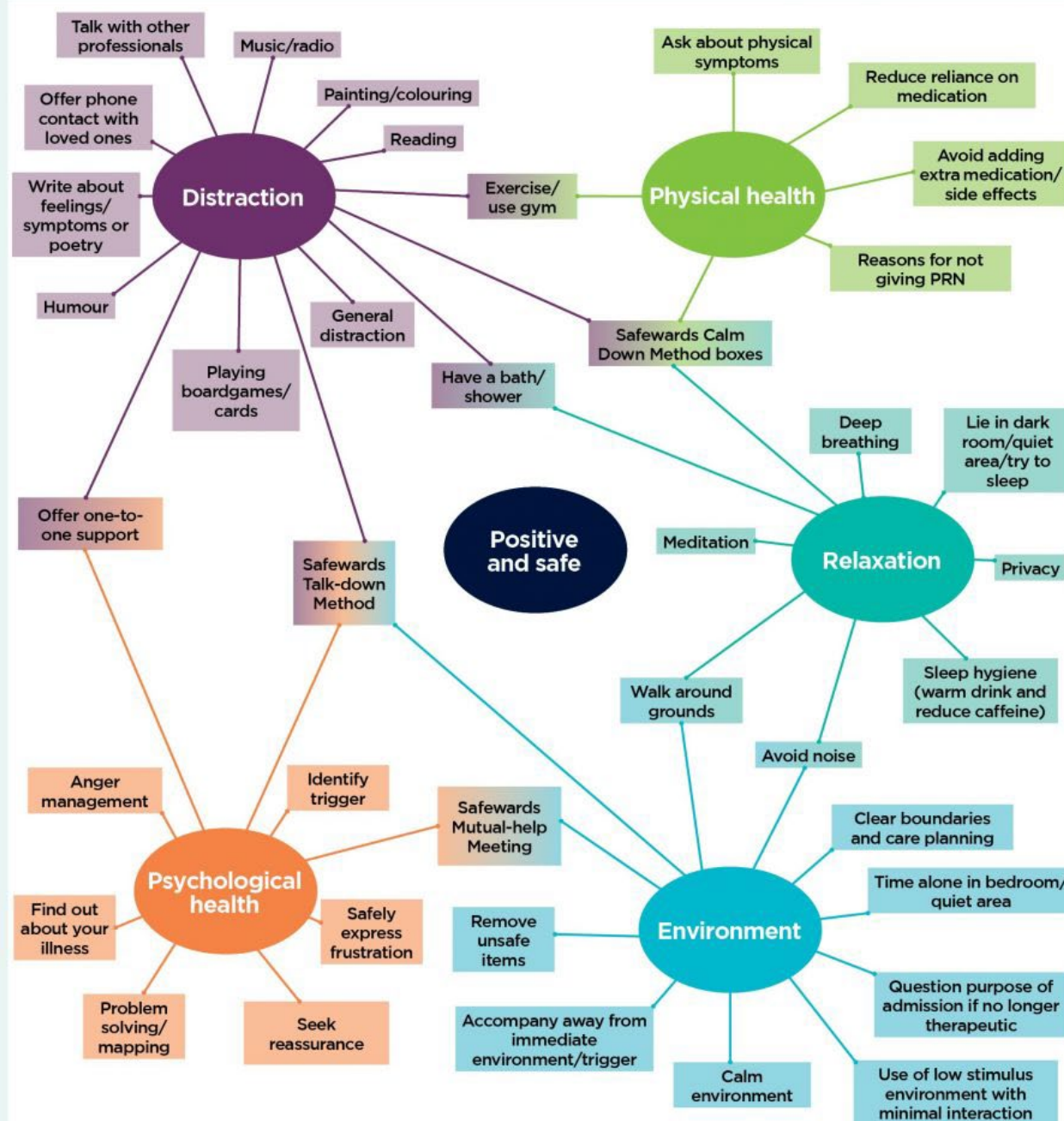
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Appendix 1: Clinical Aid Map

Interventions under the themes of physical health, relaxation, distraction, environment and psychological health

Fig 1. Map of nurse-led interventions as an alternative to PRN psychotropic medication



*PRN = *pro re nata*